



PROCEDURE

Trauma Response Emergency Department Assessment and Referral (Adult, ≥16 years old)

TARGET AUDIENCE and SETTING

Emergency Department Doctors and Nurses
Trauma Team respondents' units including Intensive Care Unit, General Surgery and Diagnostic Imaging, Pathology
Adult Retrieval Victoria (ARV)

PURPOSE

This procedure outlines the process for managing adult trauma presentations to the Emergency Department (excluding the Victorian Heart Hospital)

DEFINITIONS

BSA - body surface area
ARV - Adult Retrieval Victoria
ATS – Australasian Triage Scale

STANDARD REQUIREMENTS

When undertaking any clinical interaction with a patient, staff are expected to:

- Perform routine hand hygiene. Refer to the [Hand Hygiene Procedure](#).
- Introduce themselves to the patient and carer/ family if in attendance.
- Check patient identification. Refer to the [Patient Identification Procedure](#).
- Obtain consent as per the [Consent to Medical Treatment Procedure](#).
- Keep the patient/carers informed and involve them in decision making.
- Document interaction (such as patient review, assessments, care plans and procedures) in the appropriate location within the electronic medical record (EMR) **or** paper health record using black or blue pen; including date, time, signature and designation. Refer to [Clinical Documentation in the Health Record](#)

PROCEDURE

Patients with overt or potential trauma are assessed in line with the Victorian Trauma System Major Trauma Triage Guideline. This process steps through defined criteria identifying level of risk and defining actions aligned with the risk determined.

PROCESS

Patient presents with history of trauma including falls in adults >65

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1. Risk is assessed
2. Triage category allocated
3. Trauma Call or Trauma Alert called
4. Cubicle allocated as appropriate
5. Response to Trauma Call/Alert
6. ARV contacted if patient meets major trauma criteria within 60 minutes of diagnosis
7. Clinical assessment and management by attending team

RISK ASSESSMENT

Risk Assessment for patients presenting following trauma, including falls in adults >65 includes:

- Vital signs criteria -Australasian Triage Scale (ATS)
- High-risk criteria
- Injury pattern criteria

Where patient presenting with trauma meets the criteria in Figure 1, a Trauma Call or Trauma Alert is initiated as per Figure 2.

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Figure 1

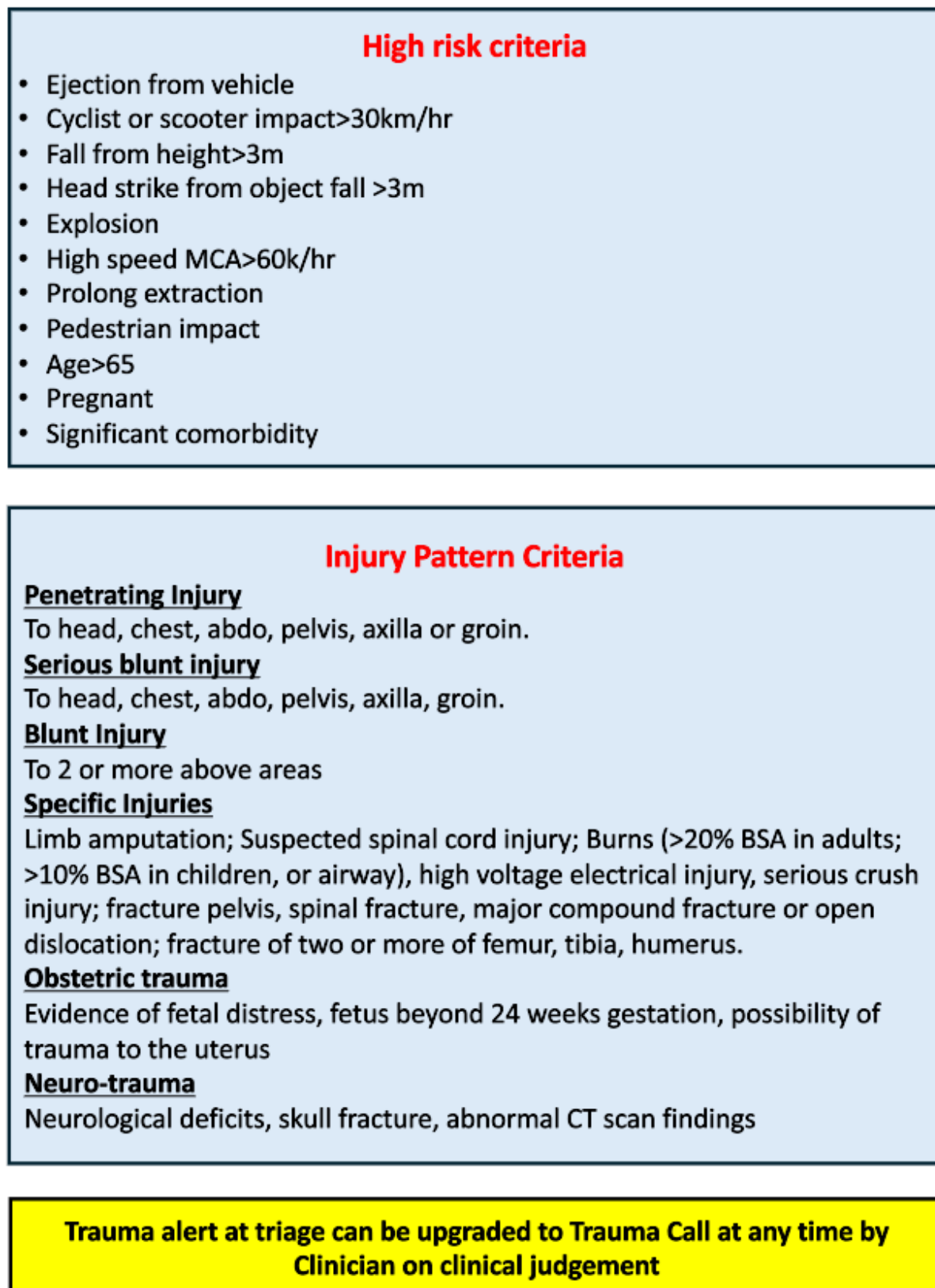
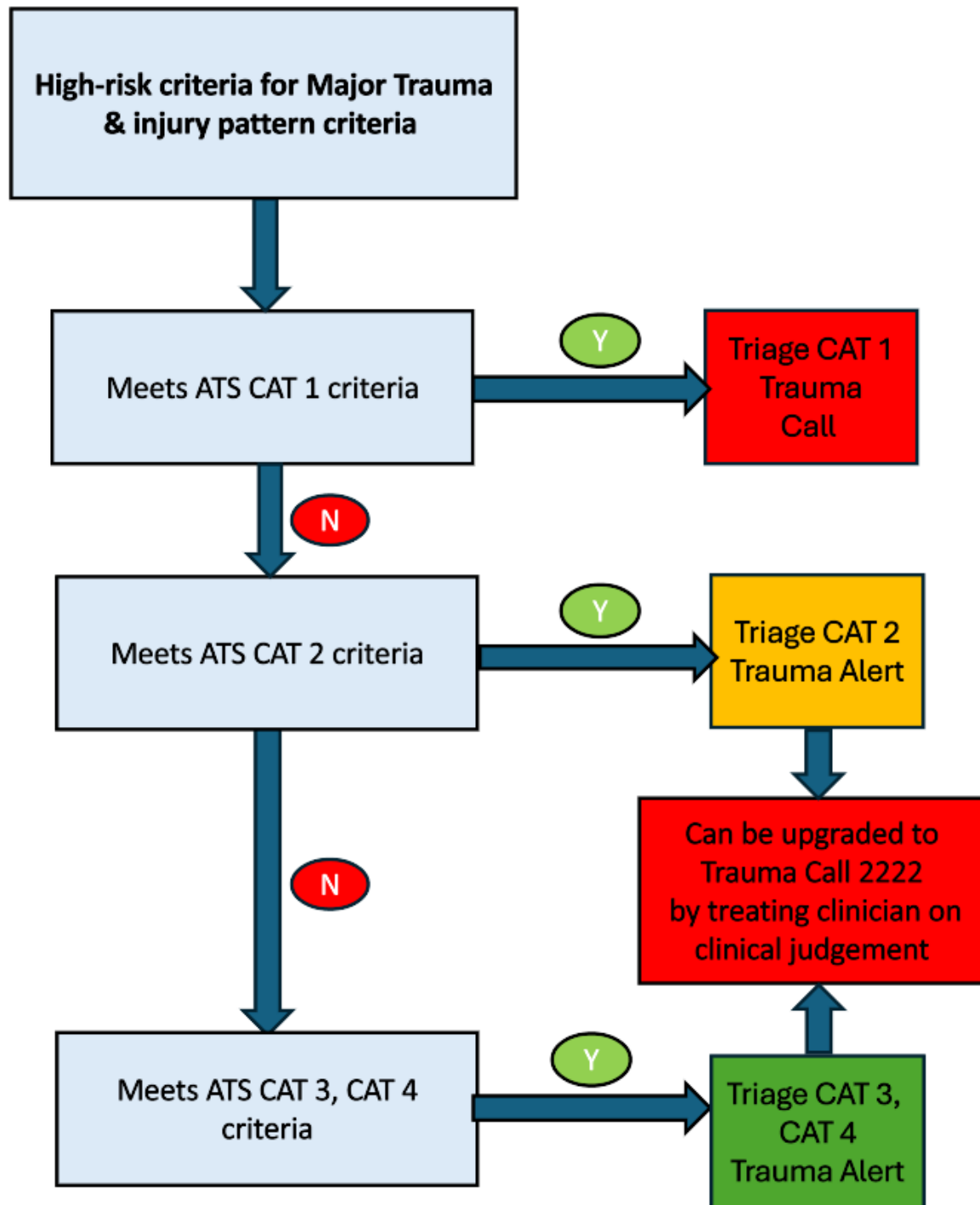


Figure 2



INITIATING TRAUMA CALL OR TRAUMA ALERT / TRIAGE WORKFLOW

Risk assessment as above determines the need for Trauma Call or Trauma Alert and triage category allocation.

Trauma Call or Trauma Alert can be made at triage, or at any other time during the patient's assessment that it becomes apparent that the patient meets the relevant criteria.

The trauma patient with high-risk criteria for major trauma and no injury pattern criteria may still be triaged as a Trauma Alert based upon the clinical judgment of the triage nurse.

Trauma Call must be activated for all cases that Ambulance Victoria pre-notify.

Trauma Call activated by:

- Call 2222,
- State: Emergency Trauma Call
- State site: Monash Medical Centre (Clayton), Dandenong or Casey Hospital and ED cubicle
- Switch will initiate the Trauma Call, similar to that of the health service MET call process
- Triage nurse to select 'Trauma Call' option during triage, to add alert banner to EMR

Trauma Alert activated by:

- Trauma Alert sent to ED team via ASCOM phones
- Triage nurse to select 'Trauma Alert' option during triage, to add alert banner to EMR

RESPONSE TO TRAUMA CALL AND TRAUMA ALERT

ED team are to respond to **Trauma Call** within the allocated ATS triage category time.

Onsite general surgical registrar attendance is expected as soon as possible and must be <10 minutes. If the surgical registrar is not available to attend in a timely manner (e.g. in theatre), ED team lead must escalate to the consultant of the Acute Surgical Unit for the day. If further escalation is required, call the Head of Unit followed by the Service Director.

When the Trauma Call is activated on pre-arrival, the team would gather in the allocated resuscitation area and prepare for the patient's arrival.

Additional specialty and health service units (e.g. anaesthetics, ICU, obstetrics, blood bank and diagnostic imaging) will be notified either prior to patient arrival, where appropriate and pre-notification has been received from ambulance services, or as clinically indicated by the team leader.

ED team are to respond to "**Trauma Alerts**" within the allocated ATS triage category time.

This is an internal ED notification process. Inpatient teams may be contacted after initial ED medical review, and the response times are expected to be that of any critical referral, as quickly as reasonably possible.

ADULT RETRIVAL VICTORIA (ARV) CONTACTED

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ARV (phone 1300 36 86 61) must be contacted within 60 minutes for adult trauma cases meeting major trauma criteria.

The Victorian State Trauma System (VSTS) provides support and retrieval services for critically ill trauma patients requiring definitive care, transfer, and management. Refer to the [inter-hospital transfer guideline](#) for evidence-based advice on the transfer of major trauma patients who present to Victorian health services.

The ED medical team leader (or delegate) will make the call to ARV. This call links the Trauma Team Leader with ARV duty consultant and the duty trauma consultant at the Metropolitan Trauma Service (MTS). The outcome of this call is either that the patient is suitable for ongoing management at Monash Health Hospital or that the patient is transferred to MTS. In the latter case, ARV will facilitate the transfer arrangements.

RELATED DOCUMENTATION

[Management of Pleural and Mediastinal Drain Tubes \(Adult\)](#)
[Isolated Blunt Chest Trauma: Management and Disposition](#)
[Management of Potential Cervical Spine Injuries in Adults](#)
[Soft Collar for Adults with Suspected Cervical Spine Injury](#)
[Emergency Department MET Responses](#)
[Appropriate CT pan scan imaging in trauma patients at Monash Health - Clinical Guideline](#)
[Monash Imaging head trauma Adult](#)
[Adult presentation to Emergency Department with minor head injuries](#)
[Trauma call Paediatric all sites](#)

KEY STANDARDS, GUIDELINES OR LEGISLATION

Victorian Trauma System Major Trauma Triage Guideline

KEYWORDS

Trauma, Emergency Department

Document governance	
Policy or Framework supported by this	High Quality Care
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